SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Panitumumab + FOLFIRI

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

RAS wild-type metastatic colorectal cancer; first-line or selected second-line · NCCP 00448 · version 5 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This combination aims to control bowel cancer that has spread and has a confirmed RAS wild-type result. The choice depends on previous treatment and your health.

Schedule
A 14-day cycle: panitumumab, irinotecan, folinic acid and fluorouracil start on Day 1, followed by a fluorouracil pump for about 46 hours. Arrange disconnection around Day 3. Continue according to benefit and tolerance.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Panitumumab
EGFR-TARGETED ANTIBODY

- ☐ Rash, dry skin, cracks and sore nails
- ☐ Eye inflammation or corneal injury
- ☐ Low magnesium, calcium or potassium
- ☐ Diarrhoea, dehydration and kidney problems
- ☐ Infusion reaction; rare serious lung or skin effects

Irinotecan
TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Discuss RAS selection, skin/nail and eye effects, electrolyte monitoring for eight weeks after completion, infusion reactions and rare serious lung/skin toxicity. Confirm DPD testing, antiemetics, atropine suitability and the separate delayed-diarrhoea plan. Explain port/PICC care and pump connection/disconnection.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00448 v5.

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